

Disorder	Treatment/ Medication
Anxiety Disorders	1. CBT - Antidepressants - Paroxetine (SSRI) - Sertraline (SSRI) 2. Benzodiazepines (GABA Agonists) *short term use ONLY* - Diazepam - Alprazolam *Social Anxiety* - B-blockers (propranolol)
Obsessive Compulsive + Related Disorders	1. Behavioral Therapy - Exposure/response prevention 2. Antidepressants - Fluoxetine (SSRI) 3. Psychosurgery - To interrupt CSTC circuit- anterior cingulate gyrus or anterior limb of internal capsule 4. Deep Brain Stimulation
Somatic Symptom + Related Disorders	1. CBT • Reduce stress • Excessive attention to bodily cues • Reinforce non-sick role behavior • Correct cognitive distortions • Address emotional basis of disorder (conversion)
Trauma & Stressor Related Disorders	General: 1. Psychotherapy - Supportive therapy, behavioral 2. Antidepressants 3. Benzodiazepines *Adjustment disorder* - Coping strategies (psychotherapy) - Support groups ○ MAY need crisis intervention (hospitalization, psych meds)

Depressive Disorders

Major Depressive Disorder

1. Psychotherapy

Antidepressants INCREASE monoamines (Serotonin + NE)

1st line SSRI's: blocks re-uptake of 5-HT

- Fluoxetine
- Citalopram
- Sertraline

Side effects: GI disturbance, sexual dysfunction

1st line SNRI's: blocks re-uptake of 5-HT & Norepi

- Venlafaxine
- Duloxetine

Side effects: GI disturbance, sexual dysfunction, + increased BP

2nd line TCA's: block synaptic re-uptake of 5HT & NE & muscarinic, histamine, α_1

- Imipramine
- Amitriptyline

Side effects: cardiac

MAOI's:

- Phenelzine
- Selegiline
- Tranylcypromine

Side effects: diet restriction to avoid tyramine induced HTN

OTHERS:

1. Esketamine Nasal Spray (NMDA receptor blocker)
 - Resistant depression, rapid onset
2. Phototherapy
 - Used for SAD
3. Brain stimulation therapy

Bipolar & Related Disorders	BIPOLAR I 1. <u>Mood Stabilizer</u> - Lithium 2. <u>Anticonvulsants</u> - Valproate/ Valproic Acid 3. <u>Antipsychotics</u> - Olanzapine - Quetiapine BIPOLAR II 1. <u>Mood Stabilizer</u> - Lurasidone - o Bipolar depression - o Taken alone, w/lithium or with anticonvulsant CYCLOTHYMIC DISORDER 1. Mood Stabilizers
Sleep Wake Disorders	INSOMNIA 1. <u>Improve Sleep Hygiene</u> - o Use bed only to sleep - o Consistent sleep schedule - o Avoid caffeine after 12 - o Lie down only when sleepy, leave after 10 min if not asleep 2. <u>Sedatives</u> - Benzodiazepines - o Diazepam ▪ Recommended SHORT term use only ▪ Induces sleep, increases sleep DURATION but lower quality of sleep (N3) 3. <u>Hypnotics</u> - Non-benzo hypnotics - o Zolpidem (fewer side effects) HYPERMORNOLENCE DISORDER 1. <u>Stimulants</u> - Methylphenidate 2. <u>Stimulant- like drugs</u> - Modafinil

NARCOLEPSY

Polytherapy

1. Stimulants
 - **Dextroamphetamine**
 2. Stimulant- like drugs
 - **Modafinil**
- +
3. Antidepressants (for cataplexy)
 - **Citalopram**

Monotherapy

1. ~~Xyrem~~- ~~Sodium Oxybate~~ (schedule 3 drug)
2. **GHB** (schedule 1 drug)
 - Both treat cataplexy & somnolence

OBSTRUCTIVE SLEEP APNEA HYPONEA

1. Continuous Positive Airway Pressure (CPAP)
 - Maintains open airway by delivering compressed air at specific pressure to nasal pillow

IMPT: * THORACIC EFFORT OCCURS*

CENTRAL SLEEP APNEA:

1. Respiratory Stimulants
 - **Acetazolamide**
 - Nocturnal O₂

IMPT: * NO THORACIC EFFORTS*

CIRCADIAN RHYTHM SLEEP-WAKE DISORDER

1. Strategic phototherapy
 - Light --> SCN --> inhibits pineal gland --> decrease melatonin --> alert

PARASOMNIAS (non-REM Sleep Arousal Disorder)

1. Benzodiazepines
 - Decrease time in SWS (N3)

PARSOMNIA (Nightmare Disorder)

1. Antidepressants
 - Decreases REM cycle

PARASOMNIAS (REM Sleep Behavior Disorder)

1. Benzodiazepines
 - **Clonazepam**

PARASOMNIAS (Restless Legs Syndrome + Periodic Limb Movements)

1. **Parkinson's drugs + Benzo's + anticonvulsants**

Paraphilic Disorder	1. <u>Aversion Therapy</u> - While engaging or thinking about ones <u>paraphilia</u>, an actual NOXIOUS stimulus is applied or aversive consequence (covert sensitization) 2. <u>Antidepressants</u> - SSRI's used to control libido, control impulses, & reduce obsessions 3. <u>Anti-androgens</u> - "chemical castration" • <u>Medroxyprogesterone acetate</u> (<u>depo-provera</u>) ○ Decreases testosterone levels & reduces sex drive (minimizes deviance)
Schizophrenia Spectrum & Other Psychotic Disorder	SCHIZOPHRENIA 1. <u>Antipsychotics</u> - D2 receptor antagonists (traditional) ○ <u>Chlorpromazine</u> (thorazine) ○ <u>Haloperidol</u> (Haldol) - Antagonists of D2, 3, 4 & 5-HTA receptors (atypical) ○ <u>Clorazil</u> (clozapine) ▪ 1-2% agranulocytosis risk ○ <u>Risperidone</u> (Risperdal) ○ <u>Olanzapine</u> (Zyprexa) 2. Supportive Interventions - Vocational rehabilitation - Individual psychotherapy - Family education 3. Educational Augmentation - Medication-refractory schizophrenia (<u>cataonic</u> type) DELUSIONAL DISORDER 1. Antipsychotics 2. Trial separation from partner 3. Increasing sources of reality testing

Neurodevelopment disorders	Attention deficit/hyperactivity disorder - Catecholamine affecting (stimulant) - Amphetamine (Adderall) - Methylphenidate (Ritaline) - Non-stimulant - Atomoxetine (Strattera) - MOA: - Increases dopamine & NE at prefrontal cortex - Appetite loss, insomnia, edginess, GI upset, potentially abused
Neurocognitive Disorders	DELIRIUM - Antipsychotics - Respiridone - Used to treat agitation, psychosis of most delirium's - Benzo's - Diazepam - Used to treat delirium caused by ALCOHOL withdrawal ALZHEIMER'S DEMENTIA - Cholinesterase inhibitors - Donepezil - Galantamine - Rivastigmine - NMDA Receptor blocker - Memantine
Sedatives: - Alcohol - Barbiturates - Benzodiazepines	ALCOHOL - Benzo's: short acting, relieve symptoms & prevent DT's & seizures - Chlordiazepoxide - Lorazepam - Diazepam - Naltrexone - OPIOD receptor ANTAGONIST that reduces pleasurable alcohol effect - Reduces rewards, cuts down on drinking

	- Acamprosate - o Mechanism unsure - o Decreased anhedonia of protracted abstinence ▪ Euthymic, decreases cravings - Disulfiram - o Inhibits aldehyde DH, causes acetaldehyde accumulation and causes toxic reaction (nausea) for 30-60 mins - o Alcohol aversion agent - o Poorly adhered to, used short term 2. BENZO OVERDOSE/TOXICITY - Flumazenil - o Benzo competitive antagonist - o High affinity for benzo binding site on <u>GABA_A</u> receptor
Stimulants: Major: - Cocaine - Amphetamines - Ecstasy - Bath Salts - Designer K2 Minor: - Caffeine - Nicotine	1. NICOTINE USE DISORDER - Nicotine replacement therapy - o Nicotine patch/gum ▪ Lowers doses of nicotine to decrease cravings/taper dose - Varenicline* (Chantix) & Bupropion - o Increases level of <u>domapmine</u> - o May cause suicidal/erratic behavior (*black box label)
Hallucinogens: Dissociative Anesthetics: - Ketamine - PCP	1. PCP INTOXICATION - Benzos/antipsychotics - Reduce environmental stimulation - Restraints if needed

Opioids: - Heroin - Morphine - Codeine - Methadone - Oxycodone - Fentanyl	1. OPIOD OVERDOSE - Naloxone ○ Short acting opioid receptor antagonist ○ For acute overdose - Abstinence therapy (Naltrexone) ○ Must be completely abstinent ○ Long acting opioid receptor antagonist ○ Usually unsuccessful b/c doesn't treat physical symptoms of abstinence - Replacement Therapy (continues for 1-2 years) ○ Methadone (full agonist) ▪ Schedule 2 Opioid drug ▪ Addictive, potentially dangerous ▪ Only available at federally regulated opioid drug treatment program □ NOT PRESCRIBED ○ Buprenorphine (partial agonist) ▪ Schedule 3 opioid drug ▪ CAN be PRESCRIBED ○ Suboxone ▪ Buprenorphine + Naloxone (only released if drug is abused)
Gambling Disorder	1. CBT- relaxation techniques 2. SSRI's, opioid antagonists 3. Gamblers Anon 4. Family therapy

Category	Specific drug	Symptoms of overdose	Withdrawal	Treatment	Notes
Sedatives	Alcohol – Least potent	Sedation, Sleepiness, ↓anxiety, Disinhibition, Impaired judgement, Slurred speech, Incoordination, Stupor, Coma, Resp. depression (resp. acidosis)	Agitation, insomnia, anxiety, ANS hyperactivity (FATAL), nausea, vomiting, hand tremor, seizures	CBT Disulfiram (Antabuse) – alcohol detox only inhibits the enzyme that breaks down acetaldehyde, after alcohol use, causes toxic reaction → Severe nausea used only for short term if person is at high risk.	Least potent – least likely to be abused Could lead to, Korsakoff amnesia
	Benzodiazepams (diazepam lorazepam) ↑ GABA	Overall: suppression of functions Other effects: Anti-convulsant, Anesthetic	Transient hallucination: can be any modality (tactile, visual or auditory) ex. Fomication: sensation of bugs crawling under the skin	Naltrexone (Revia): opioid blocker that ↓ pleasurable effects of alcohol. Helps person stop drinking after few drinks when a slip occurs.	↑S2 stage but ↓S3 stage sleep. (Used to treat Non-rem sleep arousal disorders) Mostly causes hallucination
	Barbituates - (Phenobarbital, secobarbital)	Disrupted sleep architecture Cross tolerance to other sedatives	Delirium Tremens: Delirium as part of sedative withdrawal. Severe + uncommon and associated with high mortality rate. Mostly due to alcohol use.	Acamprosate (Campral): NMDA antagonist that ↓ anhedonia of protracted abstinence and thus ↓ alcohol crave , helps a person feel euthymic so you don't need alcohol to feel good!!	High potency thus low safety margin and most likely to be abused
Other CNS depressants	Inhalants (substances with psychoactive vapors) Paint, Petrol	Sedative intoxication effects Rashy, red, and runny nose Face discoloration Chemical smell			Associated with morbidity and mortality

Major stimulants (rule out Bipolar I/ schizophrenia by drug screen)	Amphetamines Meth	Psychological: "MANIC with psychotic features" Euphoria, grandiosity, psychomotor acceleration, stereotypies, paranoia, hallucination	Symptoms of MDD with atypical symptoms: Dysphoric mood (HAS TO BE SEEN) Fatigue and psychomotor slowing Hypersomnia with vivid unpleasant dreams ↑ appetite	CBT	Meth face: redness, meth mites, keep picking skin! Meth mouth: destruction of teeth Half life ~ 12 hrs
	MDMA (Ecstasy) MOLLY Schedule 1 drug Makes you happy!!	Elevated HR, BP (life threatening), Appetite loss (treat obesity) and insomnia, Mydriasis, seizures			Mild hallucination effects, makes common things more interesting. Empathogenesis, concern about neurotoxicity Can cause hyperthermia
	Bath salts (ivory wave, purple wave, vanilla sky, bliss)				Powder form Designer drug containing amphetamine like drugs (Methylphenidate (Ritalin))
	Cocaine	Bath salts acute toxicity: agitation, paranoia, chest pain, tachycardia, hypertension, suicidality			Short half life = 30 min Perforation of nasal septum if snorted If Injection use → injection track along the vein.
Minor stimulants	Nicotine (From tobacco and cigarettes)	Premature wrinkling of skin and eyes due to Break down collagen)	Dysphoric mood, restlessness, anxiety, difficulty concentrating, irritability, anger, ↑ appetite, ↓ HR.	NRT: GUM and patches, contain small amounts of nicotine to ↓ craving. Bupropion (Zyban) – atypical anti-depressant (NE and DA reuptake inhibitor) Varenicline (chantix): partial agonist to nAChR – helps to ↑ dopamine release from nuc. Accumbens, more effective than zyban	Is not recognized by DSM-5
	Caffeine	Dose of > 250 mg/caffeine ↑ energy, insomnia nervousness, rambling thoughts, tachycardia, diuresis, GI disturbance, muscle twitches	Headache, dysphoria, fatigue, ↓ concentration		

Hallucinogens	LSD – ACID Other classic hallucinogens Mescaline psilocybin	Hallucinations (Visual mostly but poorly formed) Mind calming effects and SNS like symptoms paranoia/ agitation depending on type of hallucination Mydriasis	No withdrawal symptoms		Most potent and long lasting (effects last 8-12 hrs) Associated with flashback perceptual experiences long after LSD is metabolized. “Hallucination persisting perception disorder” Intensification of color and false perception of movement!
	Marijuana – Weed Schedule I drug Other cannabis related: Hashish	<u>Psychological:</u> Perceptual distortions (Intensification of senses, perception of slow time) <u>Physical:</u> Conjunctival injection (Red eyes) ↑ appetite, Dry mouth Rare hallucinations unless ingested!	Psychological: Irritability and nervousness Sleep disturbance Dysphoria ↓ appetite Physical: Headache, night sweats, stomach cramping, shakiness		Gateway drug Most commonly used illegal substances Shorter acting 2-4 hrs unless ingested! Associated with “amotivational syndrome”
	Phenylclidine (PCP) – angeldust (Volatile and neurological) Other similar product: Ketamine	Depersonalization Agitation, belligerence, confusion, impulsivity, unpredictability, nystagmus, hyperacusis, ↓ responsiveness to pain , ataxia, muscle rigidity, seizure and coma	None	Acute intoxication: Benzo/anti—psychotics Reduced environmental stimuli Restraints maybe required	Tablet/powder form Bitter taste

Opioids	Morphine	Pain	Vomiting	Naloxone: short acting opioid receptor antagonist for only acute OD. Abstinence based therapy: Completely go off drug and then use naltrexone (Long acting opioid receptor blocker to block opioid effects if pt does relapse, mostly unsuccessful. Replacement Therapy: Give patients a safer opioid drug (Methadone, buprenorphine) More successful Overall, due to chronic use they will have anhedonia and physical discomfort thus eventually get off the drug on their own. Mostly 1-2 yrs		
	Fentanyl	Euphoria (depending on what you are taking)	Urine			
	Hydroxycodone	drowsiness and dysphoria	Diarrhea			
	Oxycodone	Miosis (pinpoint pupil)	Sweating			
	Heroin	Unconscious	Mydriasis		<u>Very bad flu like symptoms (Not lethal)</u>	Most intense euphoria Can be smoked, snorted so not always track marks
		Respiratory depression				
	Methadone Schedule II	Can only be given out at OPT which are federally regulated for addiction treatment.				
	Buprenorphine Schedule III	Available from doctor, and can be prescribed, administered or dispensed from doctor’s office. But physicians must undergo special training. Generally, Suboxone is given out which is buprenorphine + naloxone Prevents drug abuse, when crushed or broken in half it is released.				
Gambling disorder	Persistent and recurrent problematic gambling behavior by > 4 symptoms in 12 months 1. Gambling with ↑ amounts of money to achieve excitement 2. Restless and irritable when trying to cut out or stop 3. Unsuccessful efforts to control or stop gambling 4. Preoccupied with gambling 5. Gambles when feeling distressed 6. Continues to gamble even after losing money 7. Lies to conceal extent of gambling 8. Jeopardized relationship, job, or educational or career opportunity because of gambling 9. Relies on others for money bail out Gambling behavior is not better explained by MANIC episode					